



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Clozapine: General Practitioner Shared Care

Reference #: FSFH-MENH-POL-0031

Scope

Site	Service/Department/Unit	Disciplines
Fremantle Hospital	Mental Health Service (MHS)	Medical, Nursing, Pharmacy

1. Introduction

Shared care presents an opportunity for consumers to receive the benefits of specialist intervention combined with the continuity of care and management of comorbidity provided by a General Practitioner (GP), who maintains responsibility for all aspects of the patient's health care beyond the specified chronic disease. Benefits may include reductions in hospital admissions related to specific conditions, improved patient health outcomes, higher levels of follow-up care and patient adherence to treatment.

GPs may prescribe maintenance clozapine for treatment refractory schizophrenia under the guidance of a treating psychiatrist, with the aim of optimising community-based care for stable consumers on clozapine. Clozapine is a high-risk medication, and the purpose of this document is to establish the clinical requirements for shared care of clozapine consumers between a GP in the community and tertiary MHS.

2. Terminology

Treatment Refractory Schizophrenia (TRS)	A chronic and severe mental disorder characterised by distortions in thinking, perception, emotions, language, sense of self and behaviour with a significant impact on neurological, psychological and social functioning. Schizophrenia is defined as being 'treatment refractory' following unsuccessful trials of at least two different antipsychotic medications.
High Risk Medication	Any medication that has a heightened risk of causing significant or catastrophic harm when prescribed,

	administered, or dispensed in error. It includes medications with a low therapeutic index and medications that present a high risk when administered via the wrong route.
Clozapine	An atypical anti-psychotic most effective for TRS. It improves the positive and negative symptoms of TRS with minimal extrapyramidal side effects compared to typical anti-psychotic agents. Due to possible life-threatening side-effects its use is reserved for patients who are intolerant of, or non-responsive to, two or more antipsychotic agents.
Clopine[®]	Brand name for clozapine
ClopineCENTRAL[™]	The data base and monitoring system administered by Pfizer Australia Pty Ltd. It requires all patients, prescribing doctors, dispensing pharmacists, Centre Co-ordinators and registered centres using clozapine (Clopine [®]) to be registered with the database. At the time of policy development ClopineCENTRAL [™] hold the public hospital tender for clozapine in Western Australia.
Clopine[®] Centre	Fremantle Hospital Mental Health Service (FH MHS) is the overseeing 'Clopine [®] Centre', known as "Alma St Centre" (ASC) in ClopineCENTRAL [™] .
Clopine[®] Clinic	The GP, Cockburn Wellbeing and community pharmacy will be registered as a 'Clopine [®] Clinic' of FH MHS "ASC".
Personal Support Person (PSP)	Person who is supporting someone who is experiencing mental illness. May be any of the following: family member, carer, nominated person, the parent or guardian of a child and any guardian or enduring guardian adult.
Shared Care Model	Where there is joint responsibility for planned care that is agreed between healthcare providers, the consumer and any PSP they would like to engage. The GP is at the centre of the model and there is an agreed schedule of visits with defined roles for each provider.
Clozapine Induced Gastrointestinal Hypomotility (CIGH)	Constipation is a common adverse effect of clozapine. Consumers prescribed clozapine may exhibit hypomotility or 'slow gut' which may result in severe constipation, ileus and bowel obstruction. CIGH is a highly prevalent and potentially fatal adverse effect of clozapine.
FBC	Full blood count
WBC	White blood cell count

ANC	Absolute neutrophil count
Pharmaceutical Benefits Scheme (PBS)	Under the PBS the government subsidises the cost of a list of medicines available to be dispensed to eligible consumers.
Enhanced General Practitioner Liaison (EGPL)	Service stream of the FH MHS Assessment and Treatment Team providing time limited access to a tertiary mental health service for consumers aged 18 years and above who are referred by their GP and reside in the Fiona Stanley Fremantle Hospital Group (FSFHG) catchment area. The service provides mental health advice and support by both face to face and telephone consultations to consumers and GP liaison.

3. Policy

- Not all tertiary consumers on clozapine maintenance therapy are suitable to be transferred to GP shared care.
- When considering shared care suitability, consumer characteristics must be considered as a determining factor, rather than the duration of clozapine therapy.
- FH MHS will provide guidance and support to GPs for the ongoing management and care for clinically suitable consumers on clozapine including:
 - Agreed monitoring protocols
 - Prescribing responsibilities for prophylaxis and treatment of clozapine related adverse effects
 - Required communication between service providers and the consumer
- The identified GP must register with ClopineCENTRAL™ and agree to adhere to the ClopineCENTRAL™ Protocol and the FSFHG 'Clozapine: General Practitioner Shared Care' Policy.
- GPs and pharmacies will only have access to information for consumers they are responsible for.
- Treatment supervision will be provided by the treating psychiatrist with review at regular intervals at FH MHS, at a minimum six monthly for PBS prescribing of clozapine.
- The GP is informed to forward pathology and monitoring results (e.g., ECG, metabolic monitoring, clozapine level, echocardiogram if required) to the Clinical Nurse Specialist Clozapine Coordinator (CNS CC) for collation.
- The CNS CC will identify and support GPs and / or pharmacies with Clopine® process issues and provide follow up support and consultations if required.

- Where clinical issues are unable to be resolved and or significant deterioration has occurred, FH MHS will provide a seamless transition to appropriate tertiary services.
- FH MHS and Curtin University have a Memorandum of Understanding (MOU) in place outlining the agreed process for establishing the Cockburn Wellbeing Clozapine Clinic for the provision of ongoing clozapine treatment for clinically stable consumers as part of a shared care arrangement.

4. Procedure

Clopine® brand must be prescribed for South Metropolitan Health Service (SMHS) patients – brand substitution is not allowed.

4.1. Consumer inclusion criteria

- To be eligible for GP shared care, a consumer must meet the following criteria:
 - Active with the Community Treatment Team (CTT) for a minimum of six months with low or no case management needs; stable accommodation and readily contactable
 - Clinically stable with no mental health inpatient admissions and good adherence to taking clozapine for a minimum of 12 months
 - Stable dose of clozapine for a minimum of three months or the treating psychiatrist considers clozapine dose to be stable
 - Able to independently attend blood tests, appointments, and other investigations, or adhere to the requirements of the treatment plan with a PSP when required. The PSP must be readily contactable in case of emergency.
 - The psychiatrist, pharmacist, consumer and GP agree to shared care in the community.
- Risks to be specifically considered for exclusion:
 - History of overdose or significant alert history
 - Significant current substance use
 - History of red range (**WBC < 3.0 x 10⁹/L or ANC < 1.5 x 10⁹/L**) blood results within previous 12 months
 - Consumers with benign ethnic neutropenia (BEN) who may have lower ranges for WBC and ANC approved by the Clopine® or FSFHG Haematology

- Inability to support financial cost of loss of access to voucher system e.g., Blister Packing and GP appointments.
- For GP shared care, the consumer must agree to attend:
 - Appointments with their GP every 28 days or as clinically indicated
 - Pathology for blood tests within 48 hours prior to the 28-day GP appointment and as required per protocol
 - Their nominated pharmacy to collect clozapine prescription
 - Review appointments at FHMHS at a minimum six monthly
 - And to contacting their GP if they have any concerns in between appointments particularly if they experience constipation, sore throat, fever or are feeling unwell.

4.2. GP shared care

4.2.1. Once a clozapine consumer (refer to [Appendix 1](#)) – is identified as suitable for discharge to GP shared care:

- The treating psychiatrist will write a detailed referral letter (clinical letter template in the Digital Medical Record) to provide a written clinical handover for the GP. The clinical handover will include:
 - Relevant history, e.g., physical health monitoring results
 - Recent medical reviews
 - Psychiatric history
 - Current medication and adverse reaction lists
 - Current pharmacy
 - Other relevant information specific to the consumer.
- The CTT Care Coordinator will complete relevant discharge documentation including the Care Transfer Summary in liaison with CNS CC and deactivate the consumer from the CTT
- The CNS CC will provide the GP with the GP Shared Care (GPSC) Clozapine Checklist (refer to [Appendix 2](#)) and liaise with the consumer and GP to facilitate shared care and may attend the consumers first clozapine appointment to:
 - Ensure GP is suitable for GPSC with clozapine consumer, CTT and PSP
 - Provide further verbal handover
 - Give the consumer and GP any further information required
 - Support the consumer to book monthly GP appointments

- Ensure a smooth transition to GPSC as per [Appendix 3](#) CNS CC GPSC Checklist.
- For a GP to be identified and take over clozapine management of a consumer they must:
 - Ensure four weekly GP appointments and six-monthly appointment with a EGPL Psychiatrist at FH MHS are scheduled in advance
 - During periods of leave ensure cover is provided by a Clopine® registered GP
 - Prepare a GP mental health care plan at the initial appointment if appropriate
 - Refer to [Clozapine-elearning.pdf \(health.wa.gov.au\)](#).
 - Report any adverse events to Pfizer Drug Safety as required
 - Be willing to bulk bill appointments if needed.

5. GP four weekly appointments

5.1. Physical health monitoring

- GP will ensure the patient has completed blood tests within the 48hours prior to prescription dispensing.
- GP will review blood tests and ensure that WBC and ANC are within the acceptable range. Refer to [Clopine® \(clozapine\) Haematological Monitoring](#) for frequency of monitoring.
- GP will assess for:
 - Signs of infection and or neutropenia (e.g., flu-like symptoms, mouth ulcers, sore throat, fever, malaise, and myalgia)
 - Signs of cardiac illness (tachycardia, chest pain, shortness of breath)
 - Drug interactions.
- GP will refer to the physical health parameter monitoring tools made available for reference to the local minimum standard for monitoring. (Refer to Appendix 4 Clozapine Monitoring Form Part A and [Appendix 5](#) Clozapine Monitoring Form Part B).
- GP Monitoring will include:
 - Blood Pressure, Heart Rate, Temperature, Weight, Waist Circumference
 - Caffeine consumption and smoking habits (caffeine can increase clozapine levels, whereas smoking can reduce clozapine levels)

- Glasgow Antipsychotic Side-effect Scale (GASS) for clozapine side effect tool and ask consumer about adverse effects (Refer to [Appendix 6](#) GASS for Clozapine)

Clozapine-induced gastrointestinal hypomotility (CIGH) is a highly prevalent and potentially fatal adverse effect of clozapine. Constipation must be actively assessed, managed and monitored with direct questioning regarding consumer bowel habits. (Refer to [Appendix 7](#) Constipation Assessment Tool and [Appendix 8](#) Bristol Stool Chart).

5.2. Mental health monitoring

- Review mental state and assess medication adherence.

5.3. Clozapine prescription

- Clozapine must only be prescribed in the quantity required to ensure the consumer has enough medication until their next blood test - usually 28 days.
- Clozapine may be prescribed with the PBS streamline code 4998.
- GP to provide the pharmacy with:
 - A copy of blood results (WBC, ANC). (Refer to [Appendix 9](#) Clopine[®] Blood Count Record Form), with the prescription/s or
 - The prescription/s with a note advising, see ClopineCENTRAL[™] for results

GP must not prescribe the maximum PBS quantity of 200 tablets per strength, maximum supply must be 28 days.

5.4. GP troubleshooting

- Where the patient does not have an available FBC result in the last 48 hours at the monthly appointment:
 - The GP will identify why the consumer did not have a blood test and / or reason the results are unavailable
 - If satisfied that the consumer will have their blood test the GP can continue the clozapine assessment as usual.
 - If the consumer will run out of medication before the blood test occurs the GP can apply for a “Monitoring & Supply Change” on [ClopineCENTRAL[™]](#) (apply on-line or on the phone) to allow adequate clozapine supply until the blood test is done and reviewed in consultation with the CNS CC and registered Pharmacist.

- If the consumer refuses to get a blood test in the time agreed refer to 5.5. Escalation of Care
- If the consumer does not attend an appointment for a face-to-face assessment, then the GP practice will:
 - Contact the consumer or PSP to arrange another appointment
 - Continue clozapine management if the consumer attends the appointment and receives a blood test, ensuring interruption in therapy is less than 2 days.
 - If the consumer has an interruption to therapy and does not attend an appointment as per protocol refer to 5.5. Escalation of Care
- If the consumer experiences symptoms of COVID during their four-weekly GP appointment:
 - GP practice will refer to [WA Therapy Advisory Group Advice on Clozapine Monitoring during Covid-19](#) and [ACSQHC Position Statement Clozapine in COVID-19](#)
 - GP must ensure blood sample taken immediately to assess effect on WCC and ANC and plasma clozapine levels. Path West will complete home-visit service for blood tests.
 - Monitor patient for clinical signs of increasing clozapine plasma levels (e.g., increased sedation, hypersalivation, constipation)
 - Organise pharmacy to deliver medications if measures are stable
 - Organise phone call or telehealth appointment to assess mental state if appropriate and liaise with PSP as required.

5.5 Escalation of care

- In urgent situations GP will contact emergency services and then inform ClopineCENTRAL™
- GP must request phone consult with the CNS CC or EGPL Psychiatrist (or covering FH MHS staff) for the following:
 - Significant deterioration in mental state or concerns about mental state.
 - Evidence of non-adherence with medication, blood testing or not attending appointments.
 - Blood tests are in the red range ($WBC < 3.0 \times 10^9/L$ or $ANC < 1.5 \times 10^9/L$)
 - Clozapine therapy interruption of greater than 2 days

If a consumer misses more than two consecutive days of clozapine they must be re-titrated from a low dose and EGPL must facilitate an admission for Clozapine reinduction.

- Unable to cover a clozapine appointment with a replacement GP in an acceptable time frame to meet clozapine protocol.
- Clozapine dose changes

The GP is required to consult with the EGPL psychiatrist for review if they believe a dose adjustment is indicated for any reason.

- Other situations requiring escalation include:
 - Blood test is in the Amber range (**WBC 3 – 3.5 x 10⁹/L or ANC 1.5-2.0 x 10⁹/L**) Consumer/PSP requests review
 - Consumer/PSP requests review
 - Changes to smoking and caffeine habits. Check clozapine level and consult with psychiatrist if adverse effects develop and dose needs to be reduced.
 - Planning to conceive, pregnancy or breast feeding
 - Navigating procedure for management of clozapine consumers

6. FH MHS responsibilities

- The CNS CC maintains oversight of GP managed clozapine consumers through the administration of the ClopineCENTRAL™ database and is available to provide support including but not limited to:
 - Triage calls from GPs managing clozapine and where required organise urgent contact with a consultant psychiatrist for review and advice.
 - Provide education and support on clozapine protocol to GPs & community pharmacists.
 - Manage Clopine® registration of new personnel, GPs, and pharmacies on the ClopineCENTRAL™ site.
 - Follow up on overdue blood tests.
 - Prior to the six-monthly psychiatrist review appointment, collate monitoring data (blood tests including clozapine level, metabolic screen, latest ECG and echocardiogram if required.)
 - Attend six monthly appointments with psychiatrist (where possible) and complete relevant state-wide standardised clinical documentation.

- Liaison with relevant community mental health teams regarding clozapine consumer transition and readmission pathway
- Consult with the FH Clinical Pharmacist Clozapine Coordinator
- Ensure any clinical incidents related to Clozapine Shared Care are notified in Datix Clinical Incident Management System (CIMS)
- Report any adverse events to Pfizer Drug Safety as required
- **EGPL psychiatrist will:**
 - Review the consumer six monthly as a minimum requirement and document the review in the consumers MR
 - Be available during the week for GPs to discuss any issues relevant to clozapine consumers being managed by them.
 - Report any adverse events to Pfizer Drug Safety as required
- **EGPL CNS will:**
 - Be familiar with the CNS CC role
 - In absence of CNS CC provide care to GP shared care clozapine clients under EGPL psychiatrist.
- **Readmission pathway:**
 - If the consumer is unable to be safely managed by the GP shared care arrangement within 3 months of discharge from CTT, the psychiatrist of the relevant CTT must automatically accept back care of the clozapine consumer.
 - If the consumer is unable to be safely managed by the GP shared care arrangement when it is more than 3 months from CTT discharge, the GP will refer to FH Assessment and Treatment Team and an appropriate care pathway will be determined (Refer to Appendix 10 GP Share Care Admission Pathway)

7. Community pharmacy and dispensing pharmacist

- Transfer of care to the GP will include identification of the consumer's preferred community pharmacy as this may change when the consumer no longer needs to attend the FH MHS for clozapine management.
- The community pharmacy will be advised of the transfer to GP shared care prior to the first GP appointment to ensure required registrations are in place.
- The pharmacy and all dispensing pharmacists must:
 - Be registered with ClopineCENTRAL™
 - Adhere to Clopine® protocols

- Only dispense clozapine once blood results have been reviewed AND entered on ClopineCENTRAL™
- Liaise with the GP for clozapine related matters
- Contact the patient if they do not collect clozapine when expected. If not able to contact the patient, the pharmacist will inform GP.

8. Compliance/Performance Monitoring

- The CNS CC will evaluate compliance with this policy via routine monitoring of clinical incidents.

9. Related Policy Documents

WA Health:

- [High Risk Medication Policy MP 0131/20](#)
- [Medication Chart Policy MP 0078/18](#)
- [Guidelines for the use of WA Clozapine Initiation and Titration Chart](#)
- [Clinical Incident Management Policy MP 0122/19](#)

FSFHG:

- High Risk Medications (FSH-PHARM-POL-0005)
- Medication Administration (FSFH-HW-POL-0022)
- Clozapine Therapy- Initiation and Continuation (FSH-HW-GUI-0016)
- Clinical Documentation (FSFH-HW-POL-0039)
- Clinical Handover (FSFH-HW-POL-0035)

10. Related Standards

- NSQHS Standards:
 - Clinical Governance
 - Partnering with consumers
 - Medication Safety
 - Comprehensive Care
 - Communicating for Safety
 - Recognising and responding to acute deterioration
- Chief Psychiatrist Standards for Clinical Care:
 - Care planning
 - Consumer and carer involvement in individual care

- Physical healthcare of mental health consumers

11. Bibliography

- Winckel, K, Siskind, D. Clozapine in primary care. *Australian Prescriber*. 2017; 40(6):231. <https://doi.org/10.18773/austprescr.2017.067>
- ClopineCentral™ YOUR Connection to Clopine® patient care Protocol [PFIZ4514 Clopine Central Protocol 2022 R8.pdf](#)
- Warwick, S, C.C. Managing constipation in adults. *Australian Prescriber*. 2010; 33:116-9. [10.18773/austprescr.2010](https://doi.org/10.18773/austprescr.2010). Accessed January 2024.
- Therapeutic Guidelines. Gastrointestinal.
- Stanley S, Laugharne J, Clinical Guidelines for the physical care of mental health consumers. Government of Western Australia 2009. [Clinical Guidelines for the Physical Care of Mental Health Consumers](#)
- Guidelines for the Safe and Quality Use of Clozapine Therapy in the WA health system
- NMHS Medication: Clozapine Management by General Practitioner [NMHS MH Medication: Clozapine Prescribing and Monitoring](#)
- RACGP Shared Care Model between GP and non-GP specialists for complex chronic conditions. Position Statement – February 2023.
- [Clopine® Hub Australia \(clopinehub.com.au\)](https://clopinehub.com.au)

12. Authorisation

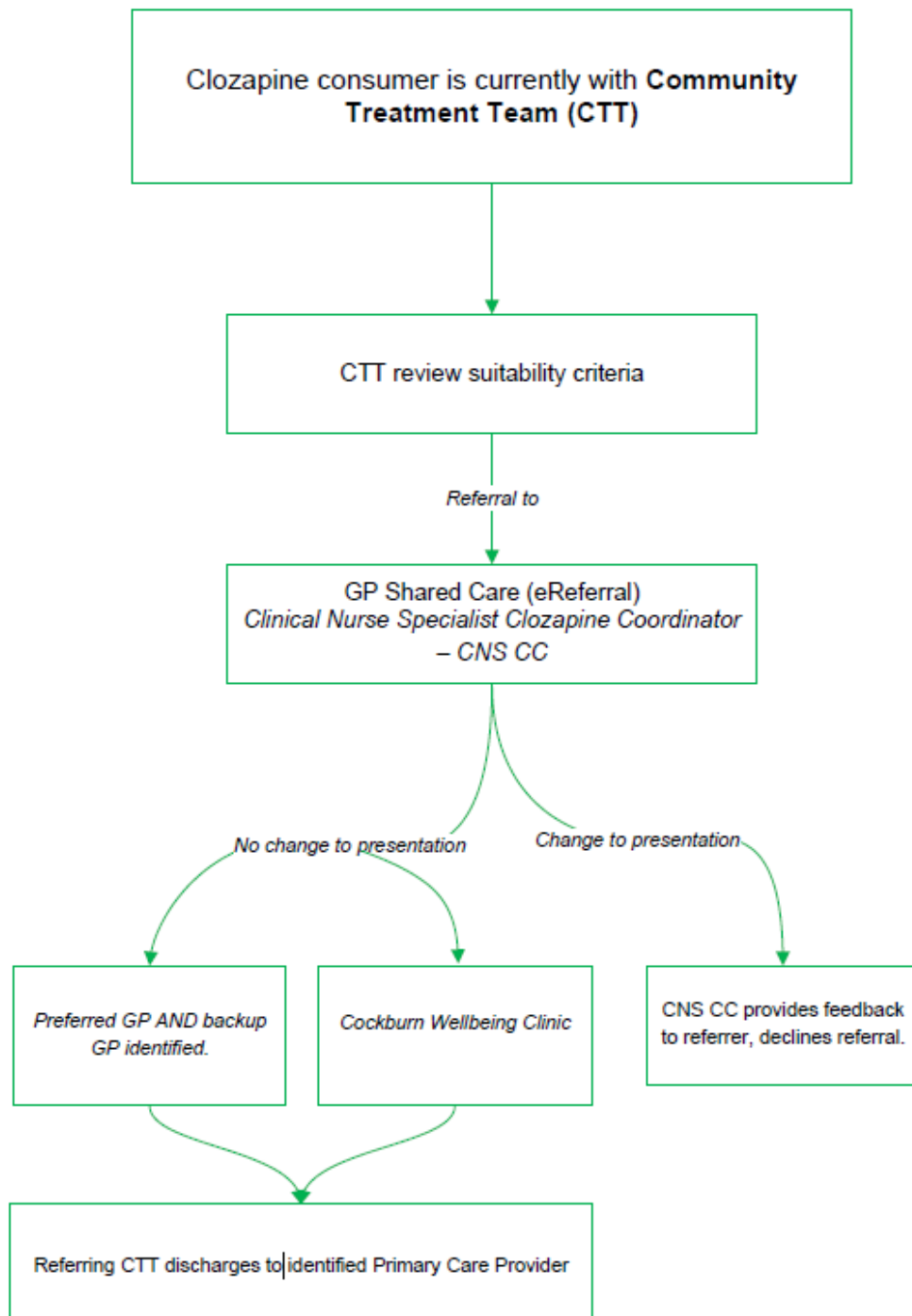
EXECUTIVE SPONSOR: Service 5 Service Director					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	01/2024	CNS, CC	Committee	Service 5 Safety Quality and Risk Committee	04/2026

13. Appendices

13.1. Appendix 1 Transition to GP Share Care Pathway



Government of Western Australia
South Metropolitan Health Service
Fiona Stanley Fremantle Hospitals Group



13.2. Appendix 2 GPSC Clozapine Checklist

GPSC Clozapine Checklist	Completed	
	Yes	No
Mental State examination: - Appearance, Behaviour, Speech, Mood, (Any changes or concerns) Psychotic symptoms - (Any changes or concerns) Risk of harm to self/others – (suicidal/self-harm or homicidal)		
Bloods taken in last 48 hours: - Green – WBC >3.5 Neutrophils >2.0 Amber – WBC 3.0-3.5 Neutrophils 1.5-2.0 Red – WBC <3.0 and/or Neutrophils <1.5 If Amber or Red Notify ClopineCENTRAL™ and seek advice from Mental Health Service		
Screen for side effects: GASS, Constipation Assessment Tool etc		
Any changes to smoking, request serum clozapine level.		
Complete Clopine® Blood Record Form and prescription for clozapine and send to pharmacy if Green .		
Ensure next appointment is made in 4 weeks' time and blood form given.		
If client does not Attend: Contact support person if not able to reach client. Check no interruption of therapy longer than 2 days. Contact mental health service if needed.		
CNS CC FH MHS (Mon – Friday 08:30 – 16:30) Email: FH.ClozapineGPSC@health.wa.gov.au	0478 853 990	
FH Switch (24/7) and ask for: - EGPL (Mon – Fri 08:30 – 16:30) - Triage (Mon – Sun 08:00 – 20:00)	9431 3333	
ClopineCENTRAL™ Support Line (Mon – Fri 09:00 – 19:30) Email: ClopineCentral@pfizer.com	1800 656 403	
Mental Health Emergency Response Line (24/7) (for after hours)	1300 555 788	

13.3. Appendix 3: CNS CC GPSC Checklist

GP Shared Care (GPSC) Arrangement CNS CC Checklist		Action Complete	
		Yes	No
Patient Arrangements	Patient has been assessed as suitable for GP Shared Care by their treating psychiatrist and meets all criteria for Clopine® Shared Care		
	Patient has been consulted regarding Shared Care arrangements and nominated preferred GP		
	Patient has given consent for the sharing of information between the MHS and the nominated GP		
GP Arrangements	The nominated GP meets all criteria for Clopine® Shared Care		
	Contact has been made with the GP to discuss the Local Health Service Clozapine protocol, the ClopineCENTRAL™ protocol and GP have agreed to provide maintenance care for the patient		
	GP is registered by Clopine® Coordinator as a Clopine® prescriber with the MHS and ClopineCENTRAL™, has allocated a coordinator of GP clinic and has back up GP for leave cover		
	Patient able to access pathology		
Pharmacy Arrangements	If a hospital pharmacy will continue temporarily dispensing, they have been consulted to discuss the patients' arrangements for collecting Clopine including date for transfer of care to community pharmacy		
	The nominated community pharmacy has registered with ClopineCENTRAL™ completed the registration form and Clopine® education including pathology requirements		
	Patient has been consulted regarding pharmacy arrangements for collecting Clopine®		
FH Arrangements	CNS CC to: ○ Update PSOLIS, PSP and GP details ○ SMS reminder for 6-month appointment GPSC - Liaison EGPL clerk ○ Alert entered in PSOLIS and file stating on GPSC ○ Added to discharged list maintained by Clozapine Coordinator for ongoing monitoring of physical health measures ○ Attend patients first GP appointment and provide appropriate resources. ○ Liaise with CTT Care Coordinator and Psychiatrist		
	CTT: ○ Psychiatrist to complete Discharge Summary on Best Practice ○ Care Coordinator to liaise with CNS CC and complete discharge NOCC, TSDP, RAMP and TOC		

13.4. Appendix 4 Clozapine Monitoring Form - Part A

Please use I.D. label or block print

DO NOT WRITE IN MARGIN

FH300200

FH00404
03/24

HCHFHFM828

FIONA STANLEY FREMANTLE HOSPITAL GROUP
CLOZAPINE MONITORING FORM – PART A
(to be used after WA clozapine initiation and titration chart)

WARD: _____ DOCTOR: _____

Psychiatrist / Treating Doctor: _____

Monitoring clinic / GP: _____

Dispensing Pharmacy: _____

SURNAME: _____ GIVEN NAMES: _____ ADDRESS: _____ TELEPHONE: _____

DOB: _____ GENDER: _____ POSTCODE: _____

UMRN: _____

Patient Contact Number: _____ Height: _____ cm

Clozapine patient number (CPN): _____ Blood Group: _____

Date clozapine initiated: ____/____/____ 18 week clozapine completion date: ____/____/____

CLINICAL REVIEW		Baseline prior to clozapine	Date	Date	Date	Date	Date	Date	Date	Date	Date	Date	Date
Baseline	Dosage (mg): AM												
	Dosage (mg): PM												
	Blood pressure – lying												
	Blood pressure – standing												
then weekly	Temperature (°C)												
first 18 weeks,	Heart rate (bpm)												
	Weight (kg)												
	Waist circumference (cm)												
then every 28 days	Cigarettes / day												
	WBC > 3.5 x 10 ⁹ /L AND NC > 2.0 x 10 ⁹ /L	Green											
	WBC > 3.0 – 3.5 x 10 ⁹ /L AND/OR NC 1.5 – 2.0 x 10 ⁹ /L	Yellow											
	WBC < 3.0 x 10 ⁹ /L AND/OR NC < 1.5 x 10 ⁹ /L	Red											
	FBC or WBC and differentials x10 ⁹ /L												
	Green, Amber or Red												
SIDE EFFECTS ENQUIRY (Indicate Yes (Y) or No (N) if side effects are present)													
Infective symptoms (inc. GI, urinary, resp)													
Signs of cardiac illness (eg. tachycardia, chest pain, SOB)													
Constipation													
Seizures													
Hypersalivation													
Nocturnal enuresis / incontinence													
Sedation													
Other side effects noted													
GASS-C form completed													
DETAILS OF PERSON COMPLETING THIS FORM													
These are suggested guidelines only, refer to the treating psychiatrist for individual monitoring requirements													
Signature or initials													
Name (Please print)													

MR 832 CLOZAPINE MONITORING FORM

13.5. Appendix 5 Clozapine Monitoring Form - Part B

Please use I.D. label or block print

FIONA STANLEY FREMANTLE HOSPITAL GROUP CLOZAPINE MONITORING FORM – PART B (to be used after WA clozapine initiation and titration chart)										SURNAME		UMRN			
WARD _____										GIVEN NAMES		DOB		GENDER	
DOCTOR _____										ADDRESS		POSTCODE			
										TELEPHONE					

Please indicate N (Normal) or A/N (Abnormal) in relevant white box once test completed (grey boxes are not mandatory but repeat tests if clinically indicated)																				
CLINICAL REVIEW REMINDERS	Baseline	1 week	2 weeks	3 weeks	4 weeks	5 weeks	6 weeks	12 weeks	6 months	12 months	18 months	2 years	2.5 years	3 years	3.5 years	4 years	4.5 years	5 years	5.5 years	6 years
Date																				
HbA1c																				
Fasting total cholesterol, LDL, HDL and triglycerides																				
Troponin																				
C-Reactive Protein																				
ECG		As clinically indicated				As indicated														
Urea and Electrolytes & Liver Function Test																				
Echocardiogram	ClopineCentral TM recommends an Echo at Baseline, 3 mths, years 1, 2, 5 and 10. Repeat Echocardiogram as clinically indicated i.e. resting tachycardia, tachypnea, shortness of breath or hypotension																			
Clozapine / Nordclozapine levels (write value)																				
Beta HCG (female)																				
Full Physical Examination booked (annually)																				
GP Letter (6 monthly)																				
Check current Care Plan (3–6 monthly)																				
DETAILS OF PERSON COMPLETING THIS FORM																				
These are suggested guidelines only, refer to the treating psychiatrist for individual monitoring requirements																				
Signature or initials																				
Name (Please print)																				

13.6. Appendix 6 GASS for Clozapine

GASS for Clozapine

Name: _____

Current Medications: _____

Date: _____

Caffeine intake:cups/day

Smoker: Y / Ncigarettes/day

Has there been a recent change in your smoking habit?: Increase/Decrease by.....cigarettes/day

This questionnaire is being used to determine if you are suffering from excessive side effects from your medication.

Please put a tick in the column which best indicates how often or how severely you have experienced the following side effects.

Over the <u>past week</u> :		Never	Once	A few times	Everyday	Tick if severe or distressing
1	I felt sleepy during the day					
2	I felt drugged or like a zombie					
3	I felt dizzy when I stood up or have fainted					
4	I have felt my heart beating irregularly or unusually fast					
5	I have experienced jerking limbs or muscles					
6	I have been drooling					
7	My vision has been blurry					
8	My mouth has been dry					
9	I have felt sick (nauseous) or have vomited					
10	I have felt gastric reflux or heartburn					
11	I have had problems opening my bowels (constipation)					
12	I have wet the bed					
13	I have been passing urine more often					
14	I have been thirsty					
15	I have felt more hungry than usual or have gained weight					
16	I have been having sexual problems					

I have also experienced:

(please write down any other side effects **OR PHYSICAL PROBLEMS OR COMPLAINTS** that you may have experienced over the **past week**)

17	
18	
19	
20	

Adapted from the Glasgow Antipsychotic Side-effect Scale© 2007 by St. John of God Hospital and South London and Maudsley Trust

Waddell, Taylor and Hynes 2012
DT,FG,AA, PH, RD comments

13.7. Appendix 7 Constipation Assessment Tool

The normal frequency of bowel motions can vary between individuals from 3 times per day to 3 times per week. Therefore, constipation should not be diagnosed based on the frequency of bowel motions alone.

Constipation is confirmed when 2 or more of the following are present.

- two or fewer motions per week
- straining at more than 25% of defecations
- a feeling of incomplete evacuation at 25% or more defecations
- hard lumpy stools (1 or 2 on the Bristol Stool Chart see Appendix 6)
- sensation of anorectal obstruction or blockage for at least 25% of defecations
- manual manoeuvres needed to facilitate at least 25% of defecations (e.g., digital evacuation, support of the pelvic floor).

Review current medication therapy and ask about the use of over-the-counter medications, complementary medicines, and laxatives to assist in identifying possible causes of constipation and determine appropriate management.

Other common issues that could be exacerbating or causing constipation should also be assessed including:

- other medications that could be causing constipation (particularly anticholinergics)
- inadequate dietary fibre
- dehydration (e.g., not drinking enough water)
- inappropriate bowel habits (e.g., ignoring the urge to defecate)
- inadequate physical activity (e.g., from decreased mobility)
- change in environment (e.g., holiday, entry into a residential aged care facility)
- painful anorectal disorders (e.g., haemorrhoids, anal fissure)
- loss of muscle power caused by other medical conditions (e.g., chronic obstructive pulmonary disease).

Immediate medical attention is required for moderate to severe abdominal pain, abdominal distension, vomiting, overflow diarrhoea and signs of sepsis.

13.8. Appendix 8 Bristol Stool Chart

Bristol Stool Chart

Type 1		Separate hard lumps, like nuts (hard to pass)
Type 2		Sausage-shaped but lumpy
Type 3		Like a sausage but with cracks on the surface
Type 4		Like a sausage or snake, smooth and soft
Type 5		Soft blobs with clear-cut edges
Type 6		Fluffy pieces with ragged edges, a mushy stool
Type 7		Watery, no solid pieces. Entirely Liquid

First published: Lewis SJ, Heaton KW (1997) Stool form scale as a useful guide to intestinal transit time. *Scandinavian Journal of Gastroenterology* 32: 920–4

Type 1–2 indicate constipation, medical team should review and provide treatment

Type 3–4 are ideal stools as they are easier to pass, and

Type 5–7 may indicate diarrhoea and urgency. Review and if indicated there may be the need to cease/reduce laxatives or consider symptoms as a sign of overflow.

13.9. Appendix 9 Blood Count Record Form

CLOPINE® (clozapine) Blood COUNT Record FORM

Blood count results can also be submitted online (www.clopine.com.au) by either the Medical Practitioner, pharmacist or Centre Coordinator.

This form is for recording the patient's WBC and neutrophil count at commencement of Clopine® therapy, during treatment and after discontinuation of therapy.

Each prescription for Clopine® must be accompanied by a WBC and neutrophil count no more than 48 hours old. Unless a current WBC and neutrophil count has been performed and assessed as satisfactory, the next prescription for Clopine® cannot be dispensed.

PATIENT Status (Tick appropriate box)
☐ ON TREATMENT

☐ DISCONTINUED

DATE DISCONTINUED: ____ / ____ / ____

• TERMINATION OF TREATMENT FORM COMPLETED?

YES/NO (please circle)

COMMENCEMENT DATE: ____ / ____ / ____

(PLEASE Print ALL DETAILS)

* Clopine® PATIENT NUMBER: _____

* DATE BLOOD TAKEN: ____ / ____ / ____ WEIGHT: ____ kg

* WBC COUNT: ____ x 10⁹/L* NEUTROPHIL COUNT: ____ x 10⁹/L

* PRESCRIBED CLOPINE® DOSAGE: ____ mg/DAY

CENTRE: _____

* MEDICAL PRACTITIONER NAME: _____

* MEDICAL PRACTITIONER SIGNATURE: _____

* PHARMACIST NAME: _____

* PHARMACIST SIGNATURE: _____

PRESCRIPTION FILLED? YES ☐ / NO ☐

DATE DISPENSED: ____ / ____ / ____

DAYS DISPENSED: _____

PHARMACY NAME: _____

13.10. Appendix 10 GP Share Care Admission Pathway



Government of **Western Australia**
South Metropolitan Health Service
 Fiona Stanley Fremantle Hospitals Group

